What the State's mental health advocate reported in 2026

Findings that bear on the Graylands Campus Project, with page references. Mount Claremont Community Network, October 2026.

The Mental Health Advocacy Service (MHAS) is the independent statutory advocate for people treated under the *Mental Health Act 2014* and people subject to the *Criminal Law (Mental Impairment) Act 2023*. Its annual report for 2025–26, dated September 2026, was submitted to the Minister for Mental Health and the Attorney-General for presentation to Parliament. Its figures cover the whole State, and many of them bear on the Graylands plan. Page numbers below are the report's printed page numbers.

+72%	**291 → 569**	**2 → 21**	**20 of 30**
serious issues raised with advocates across WA mental health services, 2025–26	safety concerns raised with advocates in one year	Community Supervision Orders in the new law's first full year	Frankland Centre beds held by people on custody orders

In short. The State's statutory mental health advocate reports serious issues across WA mental health services up 72 per cent in 2025–26, with rises in every major category, from physical abuse and sexual safety to suicidality. It describes a system marked by delay, workforce shortages and unclear responsibility between agencies, and a new law whose community pathway grew quickly in its first full year. It says more beds alone are unlikely to resolve the system's pressures. No published plan sets out the forensic beds the State needs, the role of each site, or why Graylands is the right place for this one.

Safety in mental health services is getting worse

MHAS records the serious issues people raise with its advocates across WA's mental health services, including inpatient units and licensed psychiatric hostels. In 2025–26 they rose 72 per cent, from 397 to 683 (p. 23). The rise was "not isolated to an issue type": it ran across every major category, from physical harm and sexual safety to misconduct and suicidality (p. 24).

Serious issues raised with advocates, statewide	2023–24	2024–25	2025–26
Alleged physical abuse	72	68	105
Alleged psychological or verbal abuse	15	19	52
Sexual safety	82	69	97
Alleged financial abuse	6	7	8
Potential misconduct, wilful neglect or ill-treatment	106	87	101
Other potential serious issues	48	66	135
Suicidality	14	81	185
All serious issues	**343**	**397**	**683**

Source: MHAS Annual Report 2025–26, Table Two, p. 25. The figures count issues raised with MHAS advocates, which is not every incident in the system.

> *A greater practice focus on identifying and recording serious issues is likely to have contributed to this increase, although it does not explain the overall scale of the growth.*
>
> MHAS Annual Report 2025–26, p. 23

> *While inpatient services have increasingly relied on security officers and nursing specials to manage risk, physical assaults and aggression between consumers or residents remain a significant concern including in licensed private psychiatric hostels.*
>
> MHAS Annual Report 2025–26, p. 25

> *Combined with the reported increase in physical and psychological harm, these figures raise broader concerns about the ability of some treatment environments to consistently provide safe, therapeutic and recovery-oriented care.*
>
> MHAS Annual Report 2025–26, p. 25

The largest single rise was in suicidality, from 81 to 185 matters. MHAS's category covers deaths by suicide, people expressing a clear intention to end their lives, and acts undertaken with the intent of dying. Children under 18 accounted for 41 of the 185 (p. 24). In youth services, "suicidality, significant self-harm, physical harm and sexual safety remained recurring concerns" (p. 42). Almost 15 per cent of all serious issues involved children, and a further 17 per cent young people aged 18 to 24 (p. 23).

Concerns about restraint rose from 78 to 95, and about seclusion from 30 to 38. MHAS reports that some restrictive interventions "resulted in significant injuries to consumers". It also raised concerns about chemical sedation, and cases where people needed admission to medical wards or intensive care before they could get appropriate mental health treatment (p. 43).

Safety concerns raised with advocates almost doubled, from 291 to 569. People reported feeling unsafe because of "aggression, violence or intimidation from other people receiving services". The adequacy of services' responses after incidents was "a recurring concern", and MHAS reports incidents that "were not documented or escalated", with "retrospective documentation and notification of incidents" among the outcomes its advocates secured (p. 42). MHAS's own advocates "worked in increasingly complex and high-risk environments" (p. 4). The report gives no figures on assaults against hospital staff.

If any of this raises concerns for you or someone you know, Lifeline is available 24 hours a day on 13 11 14.

What the report says about the people the system serves

MHAS describes the setting for the rise in aggression and violence in inpatient units and hostels: "rising acuity, behavioural disturbance and, in some settings, alcohol and other drug (AOD) use" (p. 25). Sexual safety matters ranged from allegations of sexual assault to sexually disinhibited behaviour and concerns about the vulnerability of consumers (p. 25). Allegations of verbal aggression and intimidation between people in inpatient and residential settings almost tripled, from 19 to 52 (p. 24).

Many people's needs cross several systems at once. MHAS worked with people whose needs crossed mental health, physical health, AOD, disability, housing, child protection and justice, and found the problems "particularly evident where mental illness intersected with disability, child protection involvement, homelessness risk or justice-system contact" (p. 47). It raised concerns about how the State's forensic disability system responds to people with complex or intersecting needs, including neurodivergent people (p. 80).

Under the mental impairment law, MHAS worked with 406 people in 2025–26 (p. 4). The most common entry point is a court raising whether a person is fit to stand trial (p. 55). Children made up about 11 per cent of those notifications: 29 notifications concerning 23 children, more than half of them for children under 16 (p. 58).

MHAS links delay to harm. Delays in assessment and treatment "were associated with increased risks of self-harm, suicidality, worsening psychosis or other symptoms and behavioural escalation", and in some matters unmet mental health needs "appeared to contribute to further offending behaviour or incidents within custodial settings" (p. 63).

What the report says about how the system is run

The Chief Mental Health Advocate writes that "longstanding issues seem to show little sign of change", and that "a critical lack of forensic beds, delayed access to assessment reports and lengthy stays in prison" threaten the promise of the new law (p. 2).

MHAS found "fragmented service delivery, delays and uncertainty about agency responsibility where multiple systems were involved". Services "sometimes disagreed about roles, funding responsibilities or appropriate support pathways", and people "could remain in hospital longer than clinically necessary" as a result (p. 47). For prisoners, access to hospital care depended on coordination between health and justice agencies, and in some cases "treatment options, admission pathways and agency responsibilities required clarification before access to care could be progressed" (p. 67). Where four agencies share responsibility for licensed psychiatric hostels, MHAS found "gaps in the collective accountability these agencies share" (p. 48).

The State's own data understates the problem. Referral and admission data "may not fully reflect demand for forensic mental health services" (p. 73). Courts did not notify MHAS of some matters, so reported figures under the new law "are likely to understate total CLMI activity" (p. 55). Efforts to move people to less restrictive settings were constrained by "workforce shortages", and limited clinician availability left some people waiting months for the fitness assessments courts need (p. 63). MHAS itself overspent its budget by 13 per cent, mainly on unfunded positions it needed to meet its legal obligations (p. 4).

The Graylands plan depends on the same agencies. The Taskforce that brought them together was dissolved in July 2023, and its functions now sit with separate agencies, with no single body accountable for the whole plan.

More people under the new law are being supervised in the community

The Criminal Law (Mental Impairment) Act 2023 took effect on 1 September 2024. It replaced indefinite detention with orders that have end dates, regular review by the Mental Impairment Review Tribunal, and a statutory path toward reintegration. 2025–26 was its first full year.

Under the Act	2024–25	2025–26
Community Supervision Orders made by the courts	2	21
Notifications of Tribunal review proceedings	50	209
People in Tribunal review proceedings	39	72
Custody orders naming an authorised hospital as the place of custody	33	38
Custody orders naming a prison as the place of custody	8	4

Source: MHAS Annual Report 2025–26, pp. 56–57, Tables 11 and 12. The 2024–25 figures run from the Act's commencement on 1 September 2024. Because some court notifications were missed, MHAS says the figures are likely to understate total activity under the Act (p. 55).

> *Although these numbers remain relatively small, and should be interpreted with caution, they represent an important shift in the operation of the reforms.*
>
> MHAS Annual Report 2025–26, p. 56

> *Increasingly, Advocates are supporting people not only through fitness proceedings but also through review hearings, rehabilitation planning, supervision arrangements and progression towards less restrictive options.*
>
> MHAS Annual Report 2025–26, p. 57

MHAS describes Community Supervision Orders as "a less restrictive pathway for suitable individuals while maintaining community safety" (p. 56). The report publishes no figures on leave of absence, or on breaches of leave or supervision conditions.

The Graylands campus already holds people under these orders. MHAS's quarterly data show 20 of the Frankland Centre's 30 beds occupied by people on custody orders under the Act. At 30 June 2026 all 13 operational beds in Dryandra Ward were occupied, and Dryandra admits only people on custody orders (p. 67). The site was chosen in 2021, on transport and access grounds, for a facility run under the old law of indefinite detention. Each leave decision is still made case by case. The framework those decisions sit in is new, and MHAS's figures show it working. No published document shows the site was reassessed against it.

Delay is the most consistent theme on the path out of custody

Delay was "the most consistent theme identified through advocacy during 2025-26", across "multiple stages of the CLMI pathway", including transition planning. Efforts to move people to less restrictive arrangements "were often constrained by broader system capacity, workforce shortages and the availability of suitable services and accommodation" (p. 63).

The previous year's report was more specific about Graylands. It found no defined pathway for returning forensic patients with a primary mental health condition to the community, and said Dryandra Ward's infrastructure "is not appropriate for the functional assessments" needed to decide whether someone can safely return. It recorded concern that the system was relying too heavily on the Graylands project to resolve these gaps (MHAS Annual Report 2024–25, p. 71).

Stage 1's sub-acute unit is built around structured community access as part of treatment. The plan for how patients move from that unit into the community, under what supervision and into what accommodation, has not been

published. MHAS records that “the paramount consideration in decision-making under the CLMI Act remains the safety of the community” (MHAS Annual Report 2024–25, p. 57).

More capacity at Graylands won’t fix this

MHAS’s own conclusion is that beds are one part of the answer.

> *Increasing bed capacity alone is unlikely to resolve broader system pressures. There is also an assumption that all prisoners require a forensic bed, whereas many prisoners’ treatment in a civil hospital would be appropriate for their inpatient care.*
>
> MHAS Annual Report 2025–26, p. 73

> *MHAS advocacy emphasised that forensic beds are critical, but they are only one part of a wider continuum of care.*
>
> MHAS Annual Report 2025–26, p. 72

The report lists what else reform requires: effective prison mental health services, timely assessment pathways, civil hospital and emergency pathways, community treatment and support, disability justice pathways, supported accommodation, step-down options and stronger cross-agency governance.

WA’s psychiatrists made the same case in July 2026, and said the redeveloped campus “will not deliver the number of beds sufficient for future needs” (RANZCP WA, July 2026).

The pressure point MHAS describes is acute, high-secure care. New Frankland admissions could go only into the eight-bed acute Acacia Ward, and only five of the Centre’s 30 beds were available for other prison referrals (pp. 67, 73). Stage 1 adds a 32-bed sub-acute unit and an 8-bed child and adolescent unit (Question on Notice 3084). The government’s forecast shows 30 acute forensic beds at the Frankland Centre every year to June 2029 (Question on Notice 3093), and MHAS lists the Frankland Centre’s own expansion as a step that follows the Graylands project (p. 67).

Taken together, the report describes problems in safety, staffing, coordination, data and the path out of custody. The Graylands plan answers one part of that, with buildings on one site chosen in 2021, before the new law took effect and before this evidence existed. Before main construction is committed, Ministers should require a published plan for the whole system: forensic need over 20 years, measured on data that captures unmet demand; the role of each site and service in the continuum MHAS describes; and an independent test of whether Graylands is the right location for the role it is being given. If that plan shows the site cannot hold what the State needs, as WA’s psychiatrists have warned, the State can still change course before main construction begins.

MHAS’s work produced “a substantial evidence-informed briefing paper on prisoners’ access to mental health care and treatment in Western Australia”, with reform options for discussion with Government (p. 72). It does not appear in the list of reports MHAS has published, checked on 15 October 2026. That paper belongs in the plan.

Questions the government has not answered

These are the risk questions the report raises for the Graylands plan.

- What whole-of-system plan for forensic mental health sits behind the Graylands project, and when will it be published?
- On what data were the 2021 bed numbers set, given MHAS's finding that referral and admission data understate unmet need, and have they been re-tested since the new law took effect?
- Has the government received MHAS's briefing paper on prisoners' access to mental health care, did it inform the Graylands plan, and will it be released?
- With serious issues up 72 per cent statewide, and aggression persisting despite more security officers, what safety model will protect patients and staff on the expanded campus?
- Graylands recorded 386 alleged assaults on nurses in 2008–09, the most of any hospital in that year's official figures. How many has it recorded each year since, and how many police attendances and Code Black and Code Grey calls?
- What safeguards will the 8-bed child and adolescent forensic unit have, given MHAS's findings on self-harm, suicidality, physical harm and sexual safety in youth services?
- What is the release, step-down and leave plan for forensic patients at the campus, how many breaches of leave conditions have there been, and how was community safety assessed for this site?
- Has any independent analysis tested whether Graylands is the right location for the role the plan gives it?

The community's ten published asks are listed at https://mtclaremontcommunity.github.io/graylandsforensic/faq.html.

Sources

Mental Health Advocacy Service, *Annual Report 2025–26*, dated September 2026. Every page cited here is in an unaltered extract: https://mtclaremontcommunity.github.io/graylandsforensic/assets/mhas-annual-report-2025-26-extract.pdf

Mental Health Advocacy Service, *Annual Report 2024–25*, tabled paper 629 (2025), WA Parliament

Mental Health Advocacy Service, list of published reports: https://www.wa.gov.au/government/document-collections/mental-health-advocacy-service-reports

RANZCP WA, Justice and Mental Health Roundtable Communiqué, July 2026: https://mtclaremontcommunity.github.io/graylandsforensic/assets/ranzcp-communique-jul2026.pdf

WA Parliament, Questions on Notice 3084 and 3093, answered 15 September 2026: https://mtclaremontcommunity.github.io/graylandsforensic/qons.html

Assaults on nurses by facility, 2008–09 (Hansard): https://mtclaremontcommunity.github.io/graylandsforensic/safety-record.html

OMID letter to residents, 14 September 2026: https://mtclaremontcommunity.github.io/graylandsforensic/omid-letter.html

Taskforce history and the site-selection record: https://mtclaremontcommunity.github.io/graylandsforensic/the-issue.html and https://mtclaremontcommunity.github.io/graylandsforensic/concerns.html

Online version of this summary: https://mtclaremontcommunity.github.io/graylandsforensic/advocate-report.html